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SciScore Report

Below you will find your SciScore report containing three tables. Your score is calculated based on adherence to scientific rigor criteria (Table 1) and identification of key biological resources (Table 2). Table 3 contains statistical tests and oligonucleotides but is not scored. If SciScore makes any mistakes, please contact us to help us learn and improve.

Table 1: Rigor Adherence Table

Ethics
Consent: Inclusion criteria were the diagnosis of acute IS according to the World Health Organization definition, 18 age >= 18 years, and provision of informed consent.
IRB: Ethics This study complies with the Declaration of Helsinki and was approved by the Ethics Committee of the Medical Faculty of the University of Wurzburg (17613).
Inclusion and Exclusion Criteria
Inclusion criteria were the diagnosis of acute IS according to the World Health Organization definition, 18 age >= 18 years, and provision of informed consent.
Attrition
25 Measurements were performed by an operator blinded to all clinical data .
Sex as a biological variable
17 Table 1 Definitions of systolic dysfunction , diastolic dysfunction , and heart failure Systolic dysfunction 17 LVEF < 52 (men) and <54 (women) Diastolic dysfunction (>=3 criteria fulfilled in patients without systolic dysfunction) 16 LAVI > 34 mLm2 OR LA area > 30 cm2 (if LAVI not available) Average Ee > 14 Lateral e < 10 cms OR septal e < 7 cms Tricuspid regurgitation maximal flow velocity > 2.8 ms Heart failure 15 Heart failure with preserved ejection fraction Heart failure with mid-range ejection fraction Heart failure with reduced ejection fraction LVEF >= 50 LVEF 40-49 LVEF < 40 At least one of the following: LAVI > 34 mLm2
Subject Demographics
Age: Inclusion criteria were the diagnosis of acute IS according to the World Health Organization definition, 18 age >= 18 years, and provision of informed consent.
Randomization
not detected.
Blinding
25 Measurements were performed by an operator blinded to all clinical data .
Power Analysis

Sample size calculation was based on previously published estimates of 10-24 . 4 , 5 , 7 , 8 We aimed to recruit 750 patients to reach prevalence estimates of 10 with a precision of 8.1-24 or of 24 with a precision of 21.1-27.2 .
Replication
not required.

Table 2: Key Resources Table

Your Sentences	REAGENT or RESOURCE	SOURCE	IDENTIFIER
Software and Algorithms			
Data analysis was performed with SAS 9.4 (SAS Institute Inc .	SAS Institute		Suggestion: Statistical Analysis System, RRID:SCR_008567(link)

Other Entities Detected

Your Sentences	Recognized Entity
Statistical Tests	
Statistical analysis According to the distribution of the variables , differences between groups were tested using the kh 2 test , Students t-test , ANOVA , Mann-Whitney U test , or Kruskal-Wallis test .	kh 2 test
	t-test
	ANOVA
	Mann-Whitney U test
	Kruskal-Wallis test

SciScore is an automated tool that is designed to assist expert reviewers by finding and presenting formulaic information scattered throughout a paper in a standard, easy to digest format. ***SciScore is not a substitute for expert review.*** SciScore also checks for the presence and correctness of several unique identifiers, including RRIDs (research resource identifiers) in the manuscript, detects sentences that appear to be missing RRIDs, and can even suggest RRIDs under certain circumstances. **All RRID suggestions should be verified;** only the author can know whether the suggestions are correct.

For a full description of scored criteria and tips for improving your score, please see https://www.scicrunch.com/sciscorereport-faq